

MEDICAL CHRONOLOGICAL SUMMARY

Plaintiff: Tyler Durden

Date of Loss: January 14, 2026

Type of Incident: Motor Vehicle Accident (Rear-End Collision)

Prepared For: Litigation / Personal Injury Review

INCIDENT SUMMARY

On January 14, 2026, Plaintiff Tyler Durden was involved in a motor vehicle collision in which his vehicle was rear-ended while stopped at a traffic signal. Emergency medical services responded to the scene. Plaintiff reported immediate neck pain, lower back pain, headache, dizziness, and left shoulder discomfort. Plaintiff was transported to the emergency department for evaluation and treatment.

CHRONOLOGICAL MEDICAL SUMMARY

January 14, 2026 – Emergency Department Visit

Facility: Mercy General Hospital

Plaintiff presented to the emergency department via ambulance following a rear-end motor vehicle accident. Complaints included:

- Cervical neck pain
- Lumbar back pain
- Headache
- Left shoulder pain
- Dizziness

Examination Findings

- Cervical paraspinal tenderness
- Reduced cervical range of motion
- Lumbar muscle spasms
- Mild left shoulder tenderness
- No loss of consciousness reported

Diagnostic Imaging

- CT scan of head: Negative for acute intracranial injury
- CT cervical spine: No fracture identified
- X-ray lumbar spine: No acute osseous abnormality

Diagnosis

- Cervical strain/sprain (whiplash injury)
- Lumbar strain
- Post-traumatic headache
- Left shoulder strain

Treatment

- Pain medication prescribed
 - Muscle relaxants prescribed
 - Discharged with instructions to follow up with primary care physician and begin conservative treatment
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January 18, 2026 – Primary Care Evaluation

Provider: Dr. Robert Miller

Plaintiff reported persistent symptoms including:

- Neck stiffness
- Lower back pain radiating into left leg
- Difficulty sleeping
- Intermittent headaches

Findings

- Continued cervical tenderness
- Positive lumbar paraspinal spasm
- Decreased lumbar flexion
- Pain rated 7/10

Plan

- Referred to physical therapy
- MRI of cervical and lumbar spine ordered

- Continued medication management
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January 25, 2026 – MRI Imaging

Facility: Advanced Diagnostic Imaging Center

MRI Cervical Spine Findings

- Straightening of cervical lordosis consistent with muscle spasm
- Disc bulge at C5-C6 with mild foraminal narrowing

MRI Lumbar Spine Findings

- Disc protrusion at L4-L5
 - Mild impingement upon thecal sac
 - Annular tear noted at L5-S1
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February 1, 2026 – Physical Therapy Initiated

Facility: Peak Rehabilitation Clinic

Plaintiff began physical therapy 3 times weekly for management of:

- Cervical pain
- Lumbar pain
- Reduced mobility
- Muscle weakness

Initial Functional Complaints

- Difficulty sitting for prolonged periods
- Pain while driving
- Sleep disturbance
- Difficulty lifting objects

Treatment Modalities

- Therapeutic exercises
- Stretching
- Manual therapy

- Electrical stimulation
 - Heat/ice therapy
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February–March 2026 – Ongoing Conservative Care

Plaintiff attended regular physical therapy sessions with partial improvement; however, persistent symptoms remained.

Ongoing Complaints

- Neck pain radiating into shoulders
- Lower back pain with intermittent numbness in left leg
- Frequent headaches
- Reduced activity tolerance

Pain levels fluctuated between 5/10 and 8/10 depending on activity.

March 20, 2026 – Orthopedic Consultation

Provider: Dr. Samantha Lee

Orthopedic evaluation performed due to persistent symptoms despite conservative treatment.

Findings

- Positive Spurling's test
- Lumbar tenderness with reduced range of motion
- Radicular symptoms into left lower extremity

Assessment

- Cervical disc bulge with radiculopathy
- Lumbar disc protrusion with radiculopathy
- Chronic post-traumatic pain syndrome

Recommendations

- Continue physical therapy
- Pain management consultation
- Consider epidural steroid injections if symptoms persist

April 5, 2026 – Pain Management Consultation

Provider: Dr. Anthony Ramirez

Plaintiff reported continued chronic pain affecting work and daily activities.

Diagnoses

- Chronic cervicgia
- Lumbar radiculopathy
- Myofascial pain syndrome

Treatment Plan

- Lumbar epidural steroid injection recommended
 - Trigger point injections discussed
 - Continued conservative care advised
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CURRENT STATUS

As of the latest documented treatment, Plaintiff continues to experience:

- Chronic neck pain
- Lower back pain
- Intermittent radicular symptoms
- Headaches
- Functional limitations affecting daily activities and occupational performance

Plaintiff remains under active medical care and may require future treatment including pain management interventions and continued rehabilitation.

SUMMARY OF INJURIES

Diagnosed Injuries

- Cervical strain/sprain
- Lumbar strain/sprain
- C5-C6 disc bulge
- L4-L5 disc protrusion
- L5-S1 annular tear
- Cervical and lumbar radiculopathy
- Post-traumatic headaches
- Left shoulder strain

Treatment Rendered

- Emergency care
- Diagnostic imaging
- Medication management
- Physical therapy
- Orthopedic evaluation
- Pain management consultation